

DISCHARGE SUMMARY

Patient Information

Patient Name:	Rohan Mehta	Admission Date:	10-Apr-2026
Age / Sex:	42 Years / Male	Discharge Date:	12-Apr-2026
Diagnosis:	Acute Viral Gastroenteritis with Moderate Dehydration	ICD Code:	ICD-10: A09
Policy No:	HLM-IND-449201	Claim ID:	CLM-20260412-ROHAN01
Consulting Doctor:	Dr. Neha Kulkarni (MD – Internal Medicine)	Reg No:	Reg No: MMC12345
Ward / Bed:	General Ward – Bed No. 12	Length of Stay:	2 Days

Chief Complaints at Admission

- Frequent loose watery stools (8–10 episodes per day) × 2 days
- Nausea and vomiting (4–5 episodes) × 1 day
- Generalized weakness, fatigue and decreased urine output
- Mild fever (Temp: 38.2°C recorded at home)
- Dry mouth, reduced skin turgor – clinical features suggestive of moderate dehydration

Diagnosis on Admission & Discharge

Primary Diagnosis:	Acute Viral Gastroenteritis with Moderate Dehydration
ICD-10 Code:	ICD-10: A09
Secondary Diagnosis:	Moderate Dehydration with Electrolyte Imbalance
Discharge Condition:	Stable – Patient is afebrile, tolerating oral fluids well

Clinical Investigations

Lab investigations were performed on **10-Apr-2026**. Key findings: CBC showed mildly elevated haematocrit (48.6%) consistent with haemoconcentration. Serum electrolytes revealed mild hyponatraemia (Na: 131 mEq/L) and hypokalaemia (K: 3.2 mEq/L). CRP mildly elevated at 22.4 mg/L. Stool routine positive for pus cells (8–10/HPF) and gram-negative rods. BUN mildly elevated (24 mg/dL), suggestive of pre-renal component due to dehydration. Findings are consistent with acute infective gastroenteritis and moderate dehydration.

Treatment Administered During Hospitalisation

Date	Treatment / Medication	Route/Dose	Indication
10-Apr-2026	IV Ringer's Lactate 500 mL × 2 bags	IV infusion	Rehydration
10-Apr-2026	Ondansetron 4 mg Inj	IV (slow push)	Antiemetic

Date	Treatment / Medication	Route/Dose	Indication
11-Apr-2026	IV Ringer's Lactate 500 mL × 2 bags	IV infusion	Continued rehydration
11-Apr-2026	Ondansetron 4 mg Inj (as required)	IV (slow push)	Antiemetic – SOS
10–12 Apr	ORS (Electral) sachet	Oral	Oral rehydration
10–12 Apr	Ondansetron 4 mg Tab	Oral – TDS	Antiemetic
10–12 Apr	Pantoprazole 40 mg	Oral – OD	Gastric acid suppression
10–12 Apr	Probiotic (S. boulardii 250 mg)	Oral – BD	Gut flora restoration
10–12 Apr	Paracetamol 500 mg (SOS)	Oral – SOS	Antipyretic

Condition at Discharge

Parameter	Status at Discharge
General Condition	Stable and comfortable
Fever	Afebrile (Temp: 36.8°C)
Vomiting	Resolved – no episodes in last 18 hours
Diarrhoea	Significantly reduced – 1–2 formed stools/day
Oral Intake	Tolerating ORS, water and light diet adequately
Hydration Status	Adequately rehydrated – good skin turgor, moist mucosae
Urine Output	Normal – approx. 1,200 mL in last 12 hours

Discharge Medications (Same as Prescription – 10-Apr-2026)

Medicine	Dose	Frequency	Duration
ORS (Electral) Sachet	1 sachet / 250 mL	After each loose stool / every 2 hrs	5 days
Ondansetron 4 mg Tab	4 mg	TDS (3× daily)	3 days
Pantoprazole 40 mg Tab	40 mg	OD – empty stomach AM	5 days
Probiotic Cap (S. boulardii 250 mg)	1 cap	BD (twice daily)	5 days
Paracetamol 500 mg Tab	500 mg	SOS – if fever >38°C	3 days

Discharge Instructions & Follow-Up

- Continue prescribed medications for the full duration; do not stop antibiotics or probiotics early.
- Diet: BRAT diet (Banana, Rice, Applesauce, Toast) + khichdi for 5–7 days; avoid spicy, fatty, raw foods.
- Hydration: Minimum 2–3 litres ORS/water daily; monitor urine colour (should be pale yellow).
- Rest adequately; avoid strenuous activities for 5 days.
- Return immediately** if: high fever (>39°C), blood in stools, severe abdominal pain, or inability to retain fluids.
- Follow-up appointment:** 17-Apr-2026 – OPD Internal Medicine, Sunrise Multispeciality Hospital, Pune.
- Repeat electrolytes and CBC at follow-up visit.

Signatures

Treating Doctor Signature

Dr. Neha Kulkarni

MD – Internal Medicine | Reg No: MMC12345

Hospital Stamp

Sunrise Multispeciality Hospital

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